



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب على عبد الظاهر عبد القادر

العنوان : العياط

الحي / المدينة : العياط

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 25-09-2026

رقم البطاقة : 27712152100649 رقم التليفون : 01205382878

العمر (تاريخ الميلاد) : 15-12-1977 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة: التوقيع:
 الاسم : د/ندى حسن الوظيفة: التوقيع:
 الاسم : د/نرمين رمضان الوظيفة: التوقيع:
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زينب علي عبد الظاهر عبد القادر National ID: 27712152100649	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Aug 752

وزارة الصحة و السكان
مديرية الشؤون الصحية بمحافظة القاهرة
جوستاف روسي انترناشيونال - مصر (EPE)

فاتورة علاج رقم ١٧٧٨٢٨٦٠

تقر مستشفى جوستاف روسي انترناشيونال - مصر (EPE) بأن أصول مستندات الصرف وميزانيتها القانونية صحيحة وعلى مسئوليتنا. وتم حفظها بغرفة الحفظ لدينا وتحت تصرف الجهاز المركزي للمحاسبات واننا لم ولن نتقدم بتلك الفاتورة لأي جهة أخرى للصرف.

اسم المريض زينب على عبد الظاهر عبد القادر الرقم القومي ٢٧٧١٢١٥٢١٠٠٦٤٩
قرار وزاري رقم ٢٠٢٥٣٦١١٣٩٥٢ بتاريخ ٢٠٢٥-٠٧-٢٨ بمبلغ ٦٨٣٢ مدة القرار ١٨٠ يوم
تاريخ الفاتورة ٢٠٢٥-٠٨-١٧ التشخيص سرطان بالثدي (مبادرة)
الإجراء زولاديكس تحت الجلد لمدة 6 شهور ودوائي وابحات الخصم (%)

مدينة	بريد	كمية	وحدة	تاريخ الصرف	ملاحظات
٥٦٦٢.٨٦	Goserolin Acetate 10.8 mg	1	اخرى	17/08/2025	
٤.٤٨	Chlorpheniramine 5mg Injection	1	ampoule	17/08/2025	
٥.٦٠	Dexamethasone sod. PO4 8mg	1	AMP / 2 ML	17/08/2025	
٤٩.٩٦	NORMAL SALINE 0.9% 50ml	2	bottel	17/08/2025	
٥٧٢٢.٩٠	الاجملي				

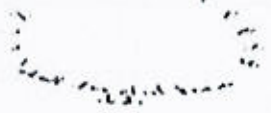
وتفضلوا بقبول فائق الاحترام،



رئيس حسابات المرضى
مصطفى ك

المراجع
طارق

الموظف المختص
ريتا كراي



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:
1- Name: زينات علي محمد الظاهر عبدالعظيم
2-ID number: 27712152100649

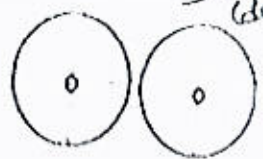
Sex:

MDT form
3- Medical number:
Female
Patient Age: 46 Y

83151607
Old Med:

2554-2023

MBC to L
Gda. no



Family history

marital status

Co morbidity

Phone

NRS Score

Menopausal status

01114547278

BMI

GROUP

A

REASON OF MDT

NEW

Date of MDT

2023-07-12 (3)

Breast Imaging:

Clinical data

Meta Workup:

DX:	ER:	PR:	Her 2 Neu:	SISH:	KI67:
	+ve	+ve	-ve		
T	N		M		

Sonoinammography

PATIENT HISTORY

acase of danoVo mets breast to bone 6/2020 , recolved 1st line AI with progression in breast
--> shifted to aromasine +palbo with SD dis.--> breast
CR--> bone
reffered for resuming treatment

MDT OUTCOME & PLAN

baseline cts & ttumor markers

continue aromasine +palbo +/- zoladex according to hormonal assay

MEDICAL ONCOLOGY	SIGNATURE	
SURGERY	02/07/23	RADIOLOGY
Psychiatric	02/07/23	PATHOLOGY
		Nutrition



Patient Name: Zainab Ali Abdel Zäher Abdel Kader	Date: 11/08/2024
Exam: C.T. CHEST WITH CONTRAST	Age: 46 Year(s)
Medical ID: 2654/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed, MD

22/08/2024 06:10 ,

ALI, ZAYNEB Patient ID: 123022626 - DOB: 15-Dec-1977 Date: 03-May-2025
Patient Lastname: Ali
Patient Firstname: Zayneb
HIS Patient ID: 123022626
RIS Patient ID: 340505
Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Mahmoud Akram
Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====



Histopathology report

Report Number: 353

Formaldehyde	التنوع	زيت على عدائنا	م الرصد
03/06/2020 12:46	الوقت	41 Y	سن
07/06/2020 13:05	وقت الاستلام		رؤى العينة
	تاريخ التقرير	صورة المرأة	المب التحليل
		صورة المرأة	نتيجة

Primary sample :

Right breast core biopsy

comment :

Gross examination :

4 cores of greyish white tissue (1.1)

Microscopic examination :

Cores of breast tissue involved by invasive cords and trabeculae of small malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab :

Comment :

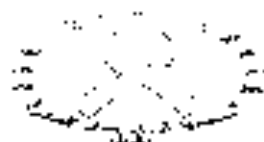
Diagnosis :

RIGHT BREAST IASIS 8 OPS / INVASIVE LOBULAR CARCINOMA, GRADE II

Pathologist

د/عبدالله بن دوي

Lab Director: د/فرزاد ماهر



Preclinical Program for Oncology
Ministry of Health and Planning

البرنامج السريري
وزارة الصحة والتخطيط



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد بن عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

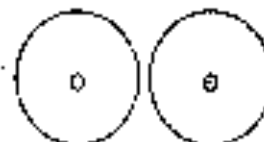
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 Y



Family history:

None

Phone:

Phone:

011 14547178

DM:

DM:

REASON OF MDT:

REASON OF MDT:

Current status:

Current status:

HRS Score:

HRS Score:

GROUP:

GROUP:

Date of MDT:

Date of MDT:

2024-12-02

Co-morbidity:

Co-morbidity:

Menopausal status:

Menopausal status:

Initial STAGE:

Initial STAGE:

Clinical data

Breast Imaging:

partial radiological response of the left breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup:

bone scan: free

ER:

ER:

PR:

HER2 Neu:

ISH:

Ki67:

ER status:

ER status:

PR status:

HER2 Neu status:

ISH status:

T:

N:

M:

1 bone

Sonomammography:

PATIENT HISTORY:

case of de novo mets breast to bone 6/2020, received 1st line AI with progression in breast

--> shifted to zoladex + aromasin + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN:

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of

99mTc MDP Revealed:

* No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	بم المريخ
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اصدار التقرير		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ تلقيح	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample:

Right breast core biopsy

Comment:

Gross examination:

4 cores of greyish white tissue

Microscopic examination:

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab:

Comment:

Diagnosis:

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. أحمد محمد

Lab Director:

د. محمد زاهر

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T. CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زينة علي عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T.CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

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- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

I-Chest:

- Still noted with stationary course of bilateral axillary level I and II enlarged lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen prominent bilateral thyroid lobes with hypodense nodules.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? Sub-serous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multi-level vertebral and left hemipelvic small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.



زینب علی حبیب الظاهر [25/9/2023]

وزارة الصحة والسكان
مركز البحوث الطبية المتخصصة
مستشفى (أريام دار السلام) - عمان

[illegible]

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023

Zohar + Arang + Ibn < 3rd

only 6/2023

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024 f

CT. 8/2024 f

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders أوامر الطبيب

المستفيد: نوع الخدمة:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		ABC 1- إعطاء 100 مل من سيريال لمدة 12 ساعة 2- إعطاء 100 مل من سيريال لمدة 12 ساعة 3- إعطاء 100 مل من سيريال لمدة 12 ساعة 4- إعطاء 100 مل من سيريال لمدة 12 ساعة 5- إعطاء 100 مل من سيريال لمدة 12 ساعة 6- إعطاء 100 مل من سيريال لمدة 12 ساعة 7- إعطاء 100 مل من سيريال لمدة 12 ساعة 8- إعطاء 100 مل من سيريال لمدة 12 ساعة 9- إعطاء 100 مل من سيريال لمدة 12 ساعة 10- إعطاء 100 مل من سيريال لمدة 12 ساعة	د. أحمد 12/12/2022

د. أحمد

Dr. Ahmad

Lab for
Ibrance 125

د. أحمد

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
المملكة العربية السعودية
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex SM on 4/1/2025 partial Radiological Report of the Breast and pelvic lymphatic nodes BS 5/2/24 CR 3/12/24 venous for Dental ② No CT ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:
رقم الملف:

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist Dear Dr, The patient has a tooth that needs to be extracted however she refused to do so and wishes to proceed with Zomeni. Dr. Fatma Ali 15/1/2025	توقيع الطبيب

→ أتمسك على
15/1/25

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R / Augmentin 1x1x30 R / Omeprazole 1x1x90 R / Calceos 1x1x90 22/6/2025 [Zoled 4 Zoled after 2 weeks] أول 4 ديسا روغيتا ٤ وردي زولاكس ٨/٨ عتاكس بنادول ٥ ٧x٥x١ سعيد C. C. A. IV سعيد محمد	

Clinical pharmacy
Zoled Zoled
رصيد
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب

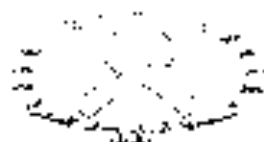


وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

أوامر الطبيب Doctor's Orders

اسم المريض ثلاثي:
عبد الرحمن

رقم الملف :



Preclinical Program for Oncology Region
Ministry of Health and Planning

البرنامج السريري لأمراض الأورام
وزارة الصحة والتخطيط



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد بن عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

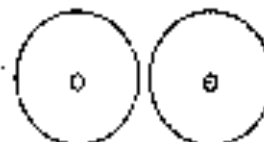
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 Y



Family history

First degree

Phone

Phone

011 14547378

DM2

DM1

REASON OF MDT

REASON OF MDT

Current status

Current status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2024-12-02

Co morbidity

Co morbidity

Menopausal

status

Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

partial radiological response of the left breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup :

bone scan: free

ER

ER

PR

HER2 Neu

ISH

Ki67

ER/PR/HER2

ER/PR/HER2

ER/PR/HER2

HER2 Neu

ISH/HER2

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

case of de novo mets breast to bone 6/2020 , received 1st line AI with progression in breast

--> shifted to zoladex + aromasine + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of 99mTc MDP Revealed:

- * No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated
Mahmoud Akram
Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	بم المريخ
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اصدار التقرير		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ تلقيح	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample:

Right breast core biopsy

Comment:

Gross examination:

4 cores of greyish white tissue

Microscopic examination:

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab:

Comment:

Diagnosis:

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. أحمد محمد

Lab Director:

د. محمد زاهر

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

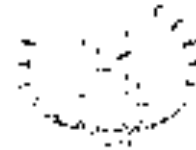
Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زسبب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

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- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

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Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

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OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023.

Zohar + Arang + Ibm < 3 ut

only 6 ut might be

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024: f

CT. 8/2024: f

اسم المريض ثلاثي: زينب عبد الله عبد الحار
رقم الملف: ٢٠٢٣ / ٢٥٥٤



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أودام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders الوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
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3/9/2023		Thin pt. met. Br. Cancer 1st line Referral for Kwik-Al-Bin to receive Aramase + Ibance + Zoladex Aramase Calcium On alpha	
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Armed Forces
Hospital

Armed Forces
Hospital

Armed Forces
Hospital

Armed Forces
Hospital

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders أوامر الطبيب

المتخصص: نوع النخبة:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		ABC 1- إعطاء 100 مل من سيريترين لمدة 12 ساعة 2- إعطاء 100 مل من سيريترين لمدة 12 ساعة 3- إعطاء 100 مل من سيريترين لمدة 12 ساعة 4- إعطاء 100 مل من سيريترين لمدة 12 ساعة 5- إعطاء 100 مل من سيريترين لمدة 12 ساعة 6- إعطاء 100 مل من سيريترين لمدة 12 ساعة 7- إعطاء 100 مل من سيريترين لمدة 12 ساعة 8- إعطاء 100 مل من سيريترين لمدة 12 ساعة 9- إعطاء 100 مل من سيريترين لمدة 12 ساعة 10- إعطاء 100 مل من سيريترين لمدة 12 ساعة	

دارم
٢٤٢

دارم
[no. 125]

Lab ok for
Ibrance 125
صالح
٢٤٢

صالح
٢٤٢

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
المملكة العربية السعودية
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex SM on 4/1/2025 partial Radiological Report of the Breast and pelvic lymphatic nodes BS 5/2/24 CR 3/12/24 venous for Dental ② No CT ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
ادارة الجودة

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist Dear Dr, The patient has a tooth that needs to be extracted however she refused to do so and wishes to proceed with zirconia. Dr. Fatma Ali 15/1/2025	توقيع الطبيب 15/1/2025

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

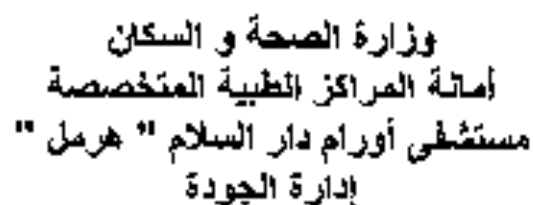
Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

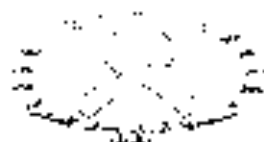
التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R / Augmentin 1x1x30 R / Omeprazole 1x1x90 R / Calceos 1x1x90 22/6/2025 [Zoled 4 Zoled after 2 weeks] أول دواء زوليدون (4) وريد زوليدون 1/8 غم بنادول 1/8 غم سعيد C. C. A. IV سعيد	

Clinical pharmacy
Zoled Zoled
Zoled Zoled
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:
رقم الملف:



Preclinical Program for Oncology Region
Ministry of Health and Planning

البرنامج السريري لأمراض الأورام
وزارة الصحة والتخطيط



ERM036726062

MDT form

A-Patient ID:

1- Name : (أحمد عبد القادر عبد الله)

Sex:

3- Medical Number:

Female

83151607

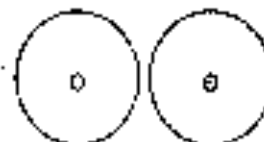
Old Med:

Date:

2554-2023

2-ID number:

Patient Age : 42 T



Family history

First degree

Phone

Phone

011 14547 178

DM2

DM1

REASON OF MDT

REASON OF MDT

Current status

Current status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2024-12-02

Co morbidity

Co morbidity

Menopausal

status

Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

partial radiological response of the rt breast malignant mass

BI-RADS 6

single left axillary indeterminate LN

Mets Workup :

bone scan: free

ER

ER

PR

HER2 Neu

ISH

Ki67

ER/PR/HER2

ER/PR/HER2

ER/PR/HER2

HER2 Neu/ISH

ISH/COMMENT

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

case of de novo mets breast to bone 6/2020 , received 1st line AI with progression in breast

--> shifted to zoladex + aromasin + palbo received palbo for 3 months only

She was irregular on ttt and kept of Aromasin only with excellent response

bone scan: CR

CT CAP: CR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of CR,

keep on Zoladex + Aromasin + Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ALI, ZAYNEB Patient ID: 123022626 DOB: 15-Dec-1977 Date: 03-May-2025

Patient Lastname: Ali

Patient Firstname: Zayneb

HIS Patient ID: 123022626

RIS Patient ID: 3405115

Date of Birth: 15/12/1977

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

=====REPORT TEXT=====

99mTc MDP Bone Scan Report (3/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I V Injection of 20mCi of

99mTc MDP Revealed:

* No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

=====REPORT TEXT ENDE=====



Histopathology report

Report Number: 355

Female	المرأة	زينة في عبداللطيف	بم المريخ
المرأة	العمر	41 Y	السن
03/01/2020 12:46	تاريخ اتمام الفحص		تاريخ اخذ العينة
07/01/2020 13:08	تاريخ التقرير	صحة المرأة	طالب التحليل
		صحة المرأة	عناية

Primary sample :

Right breast core biopsy

Comment :

Gross examination :

4 cores of greyish white tissue

Microscopic examination :

Focal of breast tissue involved by invasive cords and trabeculae of atypical malignant cells with hyperchromatic nuclei that are arranged in non cohesive pattern

Referral Lab :

Comment :

Diagnosis :

RIGHT BREAST MASS B.C.C. INVASIVE LOBULAR CARCINOMA GRADE I

Pathologist:

د. أحمد محمد

Lab Director:

د. محمد زاهر

زسب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP **Revealed:**

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mohamed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Complementary ultrasound revealed
- **Right breast:**
- No right breast residual suspicious solid mass lesions
- No pathological right axillary LNs
- Free Infra and supra-clavicular regions.
- **Left breast:**
- Single left axillary LN measured 2cm showing increased cortical thickness reaching 4mm
- No breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.

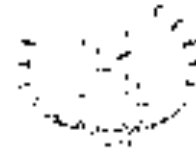
Opinion:

- Partial radiological response of the right breast malignant mass (BRADS 6)
- Single left axillary indeterminate LN for US follow up

Yours truly

Signature Dr. basma mohmaed

04/11/2024 3:24 PM



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T: CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zahab Ali Abdel Zaher Abdel Keder	Date : 11/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature Dr. Walid Mohamed; MD

22/08/2024 06:10 ,

زسبب على عبد الظاهر

^{99m}Tc MDP Bone Scan Report (29/5/2024)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Muhammed Abdeltawab, MSc



Patient Name: Zainab Ali Abdel-Zaher Abdel Kader	Date : 11/08/2024
Exam : C.T.CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 25/9/2023; the current study revealed

I-Chest:

- Still noted bilateral axillary level I and II enlarged lymph nodes.
- Clear rest of both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid lobes hypodense nodules, for US.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 17/08/2024
Exam: C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? subserous fibroid.
- No abdominal or pelvic collections.

III- The scanned skeleton:

- Still seen multiple vertebral small lytic lesions yet no vertebral structural collapse or extra-osseous soft-tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 25/9/2023.

Signature: Dr. Walid Mohamed, MD

22/08/2024 06:10



Patient Name:	Zeinab Ali Abd El-Zaher abd elkader	Patient Gender:	Female
Patient Code:	Q35077	DATE:	1/15/2023 2:43:32 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Arthritic changes of knees are noted.

N.B: focal tracer uptake is seen in the left side of the mandible likely attributed to dental problem for clinical correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous lesions.

Dr. Merna Nasef

Dr. Eman Ashraf, MD



MULTISLICE TRIPHASIC C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:-

FINDINGS:-

- The liver is mildly enlarged showing diffuse fatty texture with no suspicious enhancing hepatic focal lesions detected.
- No intra or extrahepatic biliary dilatation.
- The portal vein is patent with no thrombosis.
- Normal CT appearance of the spleen, pancreas and both suprarenal glands.
- Both kidneys are functioning and normal.
- Normal filling of the urinary bladder with normal wall thickness and no calculi.
- Still noted bulky myomatous uterus with the largest one still seen subserous arising from its anterior fundal wall, currently measures 35.5x28mm (compared to 34x27mm previously), still showing dystrophic calcification within.
- Still noted enlarged heterogeneous both ovaries, with few foci of dystrophic calcification with the largest one currently seen on the right side measuring 31x31.5mm.
- No definite sizable colonic intraluminal soft tissue lesions detected.
- No ascites.
- Bone window still reveals widespread predominantly osteolytic lesions seen randomly distributed along both axial and appendicular skeletons, likely metastatic deposits.

IMPRESSION:-

- Mild fatty hepatomegaly.
- Still noted bulky myomatous uterus with bulky heterogeneous both ovaries, for further workup.
- Multiple widespread osteolytic lesions, mostly metastatic deposits.

PATIENT NAME: ZENEB ALI ABD AL ZAHAR ABD ALKADER	PATIENT ID #: 2020130609
GENDER: Female	ORDER #: 1087477
AGE: 43 y	DATE OF SERVICE: 01-Jun-2021 04:24 PM

Report:

99MTC-MDP BONE SCAN

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:

Compared to previous bone scan dated 24/11/2020, the current study revealed rather stationary course of the previously seen wide spread osseous deposits which are seen involving the axial skeleton and proximal portions of long bones.

No newly seen lesions.

Both kidneys are seen.

Conclusion:-

Compared to 11/2020, the current study revealed rather stationary course of the wide spread metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed



Patient Name: Zainab Ali Abdel Zaher Abdel Kader	Date : 09/12/2024
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 2554/2023	Pat. ID: 42287

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Compared to previous study dated 11/08/2024: the current study revealed

I-Chest:

- Still noted with stationary course of bilateral axillary level I and II enlarged lymph nodes.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen prominent bilateral thyroid lobes with hypodense nodules.

II-Abdomen and pelvis:

- The liver is still seen mildly enlarged with diffuse homogenous low parenchymal CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still seen calcified uterine fibroid and right adnexal lesion measures about 3.3x3.5 cm containing calcific foci? Sub-serous fibroid.
- No abdominal or pelvic collections.

III-The scanned skeleton:

- Still seen multi-level vertebral and left hemipelvic small lytic lesions yet no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up status showing no gross time interval changes since study dated 11/08/2024.

Dr. Yasser Rashad, MSc

Dr. Mohamed Mohsen, M.D.

454.

15/12/2023

Premier. HBC to Bow

f 6/2023 IDC

En+ 1 PM+, 1 H2-u

1 H2-u 6/2023: 1st L AI → DP. 8/2023.

Zohar + Arang + Ibm < 3 ut

only 6 ut might be

NOT 2/12

Given 7 PCR Keep on Zohar, Arang 12/12

BS 1/2023 → 5/2024: f

CT. 8/2024: f



Doctor's Orders **أوامر الطبيب**

[illegible]

التاريخ	الوقت	الأوامر	توقيع الطبيب
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3/9/2021

This pt. met Br. Coker to have

Refused for KKR - Al - Bari

to release frame + source + holder

Answer

- Calcule

One alpha

مجلس الشورى

No.

17

1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100 101 102 103 104 105 106 107 108 109 110 111 112 113 114 115 116 117 118 119 120 121 122 123 124 125 126 127 128 129 130 131 132 133 134 135 136 137 138 139 140 141 142 143 144 145 146 147 148 149 150 151 152 153 154 155 156 157 158 159 160 161 162 163 164 165 166 167 168 169 170 171 172 173 174 175 176 177 178 179 180 181 182 183 184 185 186 187 188 189 190 191 192 193 194 195 196 197 198 199 200 201 202 203 204 205 206 207 208 209 210 211 212 213 214 215 216 217 218 219 220 221 222 223 224 225 226 227 228 229 230 231 232 233 234 235 236 237 238 239 240 241 242 243 244 245 246 247 248 249 250 251 252 253 254 255 256 257 258 259 260 261 262 263 264 265 266 267 268 269 270 271 272 273 274 275 276 277 278 279 280 281 282 283 284 285 286 287 288 289 290 291 292 293 294 295 296 297 298 299 300 301 302 303 304 305 306 307 308 309 310 311 312 313 314 315 316 317 318 319 320 321 322 323 324 325 326 327 328 329 330 331 332 333 334 335 336 337 338 339 340 341 342 343 344 345 346 347 348 349 350 351 352 353 354 355 356 357 358 359 360 361 362 363 364 365 366 367 368 369 370 371 372 373 374 375 376 377 378 379 380 381 382 383 384 385 386 387 388 389 390 391 392 393 394 395 396 397 398 399 400 401 402 403 404 405 406 407 408 409 410 411 412 413 414 415 416 417 418 419 420 421 422 423 424 425 426 427 428 429 430 431 432 433 434 435 436 437 438 439 440 441 442 443 444 445 446 447 448 449 450 451 452 453 454 455 456 457 458 459 460 461 462 463 464 465 466 467 468 469 470 471 472 473 474 475 476 477 478 479 480 481 482 483 484 485 486 487 488 489 490 491 492 493 494 495 496 497 498 499 500 501 502 503 504 505 506 507 508 509 510 511 512 513 514 515 516 517 518 519 520 521 522 523 524 525 526 527 528 529 530 531 532 533 534 535 536 537 538 539 540 541 542 543 544 545 546 547 548 549 550 551 552 553 554 555 556 557 558 559 560 561 562 563 564 565 566 567 568 569 570 571 572 573 574 575 576 577 578 579 580 581 582 583 584 585 586 587 588 589 590 591 592 593 594 595 596 597 598 599 600 601 602 603 604 605 606 607 608 609 610 611 612 613 614 615 616 617 618 619 620 621 622 623 624 625 626 627 628 629 630 631 632 633 634 635 636 637 638 639 640 641 642 643 644 645 646 647 648 649 650 651 652 653 654 655 656 657 658 659 660 661 662 663 664 665 666 667 668 669 670 671 672 673 674 675 676 677 678 679 680 681 682 683 684 685 686 687 688 689 690 691 692 693 694 695 696 697 698 699 700 701 702 703 704 705 706 707 708 709 710 711 712 713 714 715 716 717 718 719 720 721 722 723 724 725 726 727 728 729 730 731 732 733 734 735 736 737 738 739 740 741 742 743 744 745 746 747 748 749 750 751 752 753 754 755 756 757 758 759 760 761 762 763 764 765 766 767 768 769 770 771 772 773 774 775 776 777 778 779 780 781 782 783 784 785 786 787 788 789 790 791 792 793 794 795 796 797 798 799 800 801 802 803 804 805 806 807 808 809 810 811 812 813 814 815 816 817 818 819 820 821 822 823 824 825 826 827 828 829 830 831 832 833 834 835 836 837 838 839 840 841 842 843 844 845 846 847 848 849 850 851 852 853 854 855 856 857 858 859 860 861 862 863 864 865 866 867 868 869 870 871 872 873 874 875 876 877 878 879 880 881 882 883 884 885 886 887 888 889 890 891 892 893 894 895 896 897 898 899 900 901 902 903 904 905 906 907 908 909 910 911 912 913 914 915 916 917 918 919 920 921 922 923 924 925 926 927 928 929 930 931 932 933 934 935 936 937 938 939 940 941 942 943 944 945 946 947 948 949 950 951 952 953 954 955 956 957 958 959 960 961 962 963 964 965 966 967 968 969 970 971 972 973 974 975 976 977 978 979 980 981 982 983 984 985 986 987 988 989 990 991 992 993 994 995 996 997 998 999 1000 1001 1002 1003 1004 1005 1006 1007 1008 1009 1010 1011 1012 1013 1014 1015 1016 1017 1018 1019 1020 1021 1022 1023 1024 1025 1026 1027 1028 1029 1030 1031 1032 1033 1034 1035 1036 1037 1038 1039 1040 1

$\frac{1}{\sqrt{2}} \begin{pmatrix} 1 & i \\ 0 & 1 \end{pmatrix}$

Figure 1

Math. Ann. (3)
1913, 123

Lib

1. 15-10-15

وَأَمَّا مَنْ أَتَىٰ عَلَىٰ الْغَدَقَاتِ فَنَسِيَ

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
المملكة العربية السعودية
مستشفى اورام دار السلام " هرمل "
إدارة الجودة

٤٥٦

Doctor's Orders الاوامر الطبية

التشخيص: نوع التقنية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		9/12/2024 - MDT 2/12/2024 Review CR Keep on Zoladex + Arimidex, Zoladex SM on 4/11/2024 partial Radiological Report of the Breast and pelvic lymphatic system BS 5/12/24 CR 3/12/24 venous for Dental ② No CT ③ Bone Scan	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:

رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "
إدارة الجودة

Doctor's Orders أوامر الطبيب

التشخيص: نوع النخبة:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		أرغام زينة كالير بوسولم سوزنة د. محمد عبد الله For Dental center & Consider Zomela أرغام زينة 1/5 بوسولم د. محمد عبد الله 11/10/2011	

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "
ادارة الجودة

Doctor's Orders اوامر الطبيب

التشخيص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
		Dentist Dear Dr, The patient has a tooth that needs to be extracted however she refused to do so and wishes to proceed with zirconia. Dr. Fatima Ali 15/1/2025	توقيع الطبيب

→ أتمسك على
15/1/25

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

اسم المريض ثلاثي:
 رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل"
إدارة الجودة

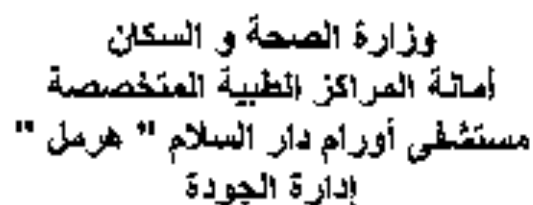
Doctor's Orders الأوامر الطبيب

الفحص: نوع التغذية:

التاريخ	الوقت	الأوامر	توقيع الطبيب
22/6/2025		Done Scan → CR (No active lesion) R / Augmentin 1x1x30 R / Omeprazole 1x1x90 R / Calceol 1x1x90 22/6/2025 [Zoled 4 Zoled after 2 weeks] أول دواء زوليدون (4) وريد زوليدون 1/8 غطاء بنادول 1/8 غطاء سعيد C. C. A. IV سعيد	

Clinical pharmacy
Zoled Zoled
رصيد
C. C. A. IV

جميع الأوامر الطبية لا بد أن تكون واضحة وموقعة من الطبيب



اسم المريض ثلاثي:
رقم الملف: